جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : سماح ابراهيم عبدالتواب سيد
المحافظة : الجيزه **الحي / المدينة :** الوراق **العنوان :** 0
رقم البطاقة : 28002282102445 **رقم التليفون :** 01127548650 **تاريخ تسجيل الإستمارة :** 21-10-2026
العمر(تاريخ الميلاد) : 28-02-1980 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الابحاث و الاشعات [7954] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: سماح ابراهيم عبدالتواب سيد
National ID: 28002282102445

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Further investigations and imaging**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين





National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

GUSTAVE ROUSSY

Patient Name	: الأستاذه / سماح ابراهيم عبد التواب	Request No.	: 13005364
Patient No.	: 28002282102445	Request Date	: 09/03/2026
Path No.	: 644/26	Verification Date	: 17/03/2026
Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data: Right breast cancer, Post-NACT.
Nature of specimen: Right breast conservative surgery & Sentinel lymph nodes biopsy.

Gross Examination:
- Right breast conservative surgery specimen measured 08x06x3 cm, with covering skin ellipse 4.5x1.5 cm.
• Serialling showed a wire guided clip inserted tumor bed site measured 4.5x2x2 cm, located 3 mm from medial margin, 1cm from lateral margin, 1.5 cm from superior margin, 0.8 cm from inferior margin, 1 cm from skin & 0.5 cm from the deep margin.
- Sentinel lymph nodes: Fibrofatty tissue piece measured 6x5x1 cm, entangling 5 lymph nodes, the largest measured 1.5x1x1 cm.
- 12 Blocks:
1:5- TBS., 6- Skin., 7- Medial margin., 8- Lateral margin., 9- Superior margin., 10- Inferior margin., 11- Deep margin., 12- SLNs.
** Gross was done by; Dr. Duaa Mohamed.

Technique:
- Sections prepared from paraffin block (Deep margin, 11) for immune-staining using Bench Mark ULTRA system for;
- P63 clone (4A4), CK5/6 clone (D5/16B4).

Frozen (Intra-operative) Results:
- Free surgical margins.
- Five free sentinel lymph nodes, SLNs (0/5).
* Frozen section was examined by; Dr. Walaa Ibrahem.

Microscopic examination:
- Residual 30% High & intermediate grade ductal carcinoma in situ (Solid pattern with

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY

Patient Name	: الأستاذة/ سماح ابراهيم عبد التواب	Request No.	: 13005364
Patient No.	: 28002282102445	Request Date	: 09/03/2026
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Sex	: Female	Financial Category	: جوستاف روسي - مبادره
Age	: 46 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

comedo type necrosis).
- No residual viable invasive component.
- Angioinvasion: Absent.
- Additional findings: Effect of therapy (Fibrosis, hyalinosis & microcalcification).
- Surgical margins:
* Free surgical margins (Deep margin showing only atypical ductal hyperplasia, proved by (P63 & CK5/6) which highlighted myoepithelial layers around two suspicious ducts measuring less than 2 mm, with few scattered cells expressing P63 inside the proliferation).
- Sentinel lymph nodes: Five free sentinel lymph nodes, (0/5).
• Histologic response to therapy: Pathologic complete response to therapy.

Diagnosis:
Right breast cancer (Post-NACT), Right breast conservative surgery & Sentinel lymph nodes biopsy:
- Residual 30% High & intermediate grade ductal carcinoma in situ (Solid pattern with comedo type necrosis).
- No residual viable invasive component.
- Surgical margins:
* Free surgical margins (Deep margin showing only atypical ductal hyperplasia, proved by (P63 & CK5/6) which highlighted myoepithelial layers around two suspicious ducts measuring less than 2 mm, with few scattered cells expressing P63 inside the proliferation).
- Sentinel lymph nodes: Five free sentinel lymph nodes, (0/5).
• Stage: ypTis, ypN0 (sln).

Signature
Dr. Walaa Ibrahim Ghoneim.
Dr. Duaa Mohamed.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

02/18/2026

MDT FORM

Name:	Samah Ibrahem Abd Eltawab	**MRN:**	1657
Age:	46 Y	**ID:**	
Menopausal Sta	AMMENORRHEA INDUCED BY CHEMO	**Phone:**	01127548650
NRS SCORE:		**Marital Status:**	MARRIED
BMI:		**Family History:**	-VE
CO Morbidity:	M:FREE IUD FOR CONTRACEPTION	**Reason Of MDT:**	POST NACT

Clinical Data:

BX: IDC II

ER: 7/8 **PR:** 6/8 **HER2:** 10% +3 **KI67:** 25

Sonomammography:
ACR B: INITIAL RT. MASS AT 10 OCLOCK 2CM FROM NIPPLE +2 PATHOLOGICAL LN
BSM POST:COMPLETE RADIOLOGICAL RESPONSE

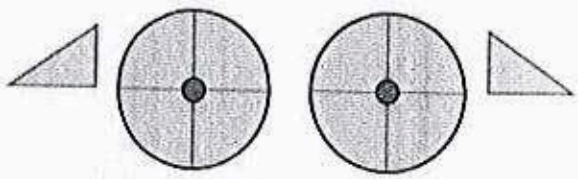
Mets Workup:
CT CAP:FREE

History:
INITIALLY T2 N1 POST NACT LAST CYCLE 9/1

Recommendations:
FOR POSSIBILITY OF RT BCS +SLNB AFTER stereotactic WIRE LOCALISATION

MDT Outcome & Plan
for BCS+ SLNB after wire localization

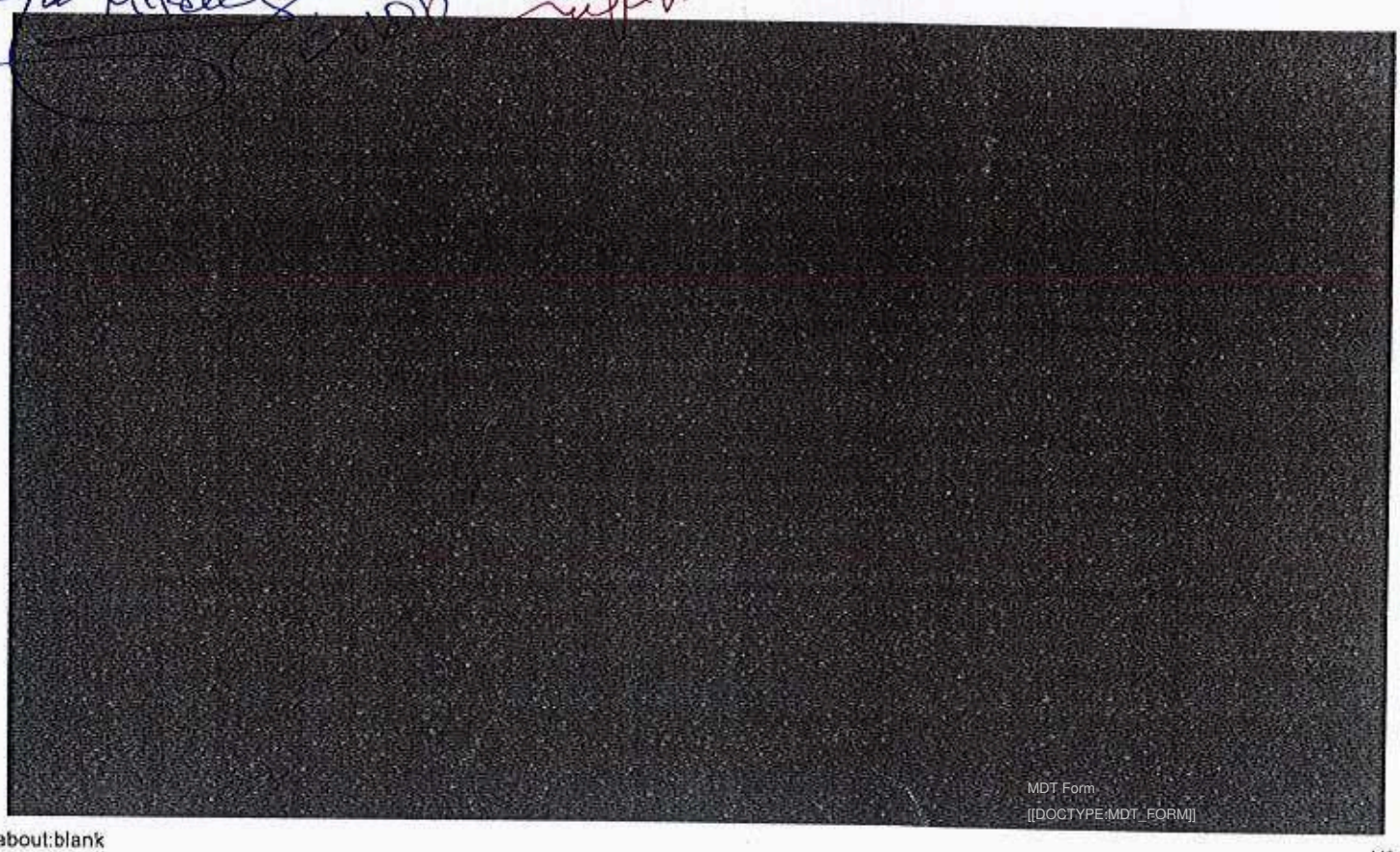
R L



TNM

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
د/ محمد كيلاني د/ ندي حسن	د/ احمد نجيب	د/ امل حجازي			



Patient Name	: الأستاذه / سماح ابراهيم عبد التواب	Request No.	: 13002537
Patient No.	: 28002282102445	Request Date	: 23/06/2025
Path No.	: 2106/25	Verification Date	: 29/06/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 45 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical Data: Right breast UOQ suspicious hypoechoic lesion.

Nature of Specimen: Tru-cut biopsy, collected at 22/06/2025.

Gross Examination:
- Multiple grey white tissue cores up to 1 cm long. Totally submitted.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), Ki67 clone (30-9).

Microscopic Examination:
* Histological type: Invasive duct carcinoma.
* Grade 2 (differentiation 3, nuclear pleomorphism 2, mitosis 2).
* DCIS: Not detected.
* Angioinvasion: Not detected.
* Perineural invasion: Not detected.
* Necrosis: Focally detected.
* IPT (Positive control showed appropriate reactivity):
- ER: Strong nuclear reaction in 50% of tumor cells (score 7/8).
- PR: Strong nuclear reaction in 20% of tumor cells (score 6/8).
- Her2: Strong complete membrane reaction in more than 10 % of tumor cells.
- Ki67 %: 25%.

Diagnosis:
Right breast mass, Tru-cut biopsy:
- Invasive duct carcinoma, Grade 2.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / سماح ابراهيم عبد التواب	Request No.	: 13002537
Patient No.	: 28002282102445	Request Date	: 23/06/2025
Path No.	: 2106/25	Verification Date	: 29/06/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 45 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

IPT:
- ER: Positive (score 7/8).
- PR: Positive (score 6/8).
- Her2: Positive (score 3+).
- Ki67 %: 25%.

* Scoring system: "ER, PR"
ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66%
and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, week, 2, intermediate; and 3, strong). Total score from 0 to 8.
* Scoring system: "Her2"
Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013.IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells.
IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within >10% of
the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is complete, intense > 10% of tumor cells.

Signature
Dr. Walaa Ebrahem Ghoneim.
Dr. Heba Hassan.

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

GUSTAVE ROUSSY
INTERNATIONAL CANCER CAMPUS

Patient Name	: الأستاذه / سماح ابراهيم عبد التواب	Request No.	: 13002537
Patient No.	: 28002282102445	Request Date	: 23/06/2025
Path No.	: 2106/25	Verification Date	: 29/06/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 45 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: Mrs.Mayada Hegazy

Pathology Report

Verified by :
Dr.Duaa Mohamed

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Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

DMS 170V

Name: Mrs. Samah Ibrahiem Abdultrawab Saied
Date: 2026-01-20

Bilateral Digital soft tissue mammography with high resolution breast sonography revealed:

Clinical summary: Right breast triple positive molecular subtype breast cancer, underwent NACT, follow up study.

Findings:

Right breast.

- Scattered fibro-glandular densities. (ACR b)
- UOQ, posterior third, tissue marker with surrounding subtle architectural distortion with no speculations, masses or asymmetry.
- **Ultrasound** demonstrated 10 o'clock, posterior third area of granulation tissue with no vascularity within.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Left breast.

- Scattered fibro-glandular densities. (ACR b)
- No speculations, masses, distortion, asymmetry or suspicious microcalcifications.
- **Ultrasound** demonstrated no solid or cystic lesions.
- Normal skin/nipple/areola complex.
- Non-specific axillary LNs.

Conclusion: -

Right breast (BIRADS 6): complete radiological response of the previously reported malignant process.

Left breast (BIRADS 1): Normal study.

Best Regards,
DR, Amal Hegazi, M.Sc.

BIRADS classification of breast lesions according to the American College of Radiology used for breast assessment combined with recommendations:

Category & recommendations	Suspected Cancer Risk
BIRADS 0: Non-informative study.	
BIRADS 1: Normal study, routing screening	0 %
BIRADS 2: Benign findings, routine screening	0 %
BIRADS 3: Probably benign, for close follow up.	2 %
BIRADS 4: Probably malignant, for biopsy.	50 %
A: mild suspicion.	
B: moderate suspicion.	
C: high suspicion.	
BIRADS 5: Malignant, for biopsy.	98-100%
BIRADS 6: Pathologically proven malignant	

Pn CBS + SNLB
after wire localization

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Samah Ibrahim Abd Al Tawab Sayed	Date : 29/05/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 45 Year(s)
Medical ID: 28002282102445	Pat. ID: 99892

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UOQ spiculated irregular mass lesion, of high density measured 39x35 mm(measured in CC view) with overlying microcalcifcations.
- Complementary ultrasound revealed
- **Right breast:**
- Right breast irregular spiculated hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 10 o'clock position 2cm from the nipple, 0.3cm from the overlying skin, measuring 2.8x5cm, consistent to the palpable lump.
- Two Pathological right axillary lymph nodes level I are seen with muffled hila, measuring 1.1x1.5cm, with cortical thickness reaching 5mm
- Free right supra and infraclavicular regions
- **Left breast :**
- No solid or cystic mass lesions
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific left axillary LN.

Opinion:

- Right breast UOQ unifocal malignant looking mass lesion (BIRADS 5) for histopathological correlation.
- Pathological right axillary LNs level I .
- Benign findings left breast (BIRADS 2)

Yours truly

Signature Dr. Basma Mohamed

29/05/2025 2:40 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

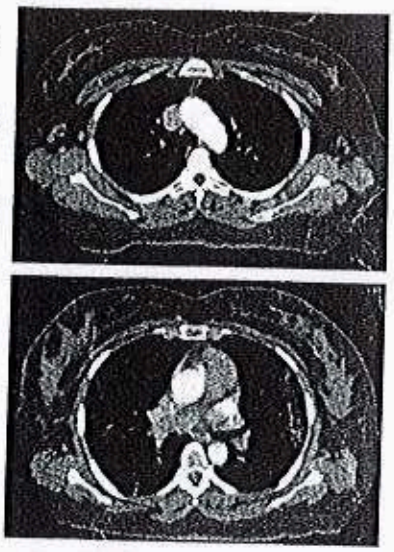
Patient Name: Samah Ibrahim Abd Al Tawab Sayed	Date : 09/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 45 Year(s)
Medical ID: 1919/2025	Pat. ID: 99892

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST: -

Clinical History:

- ***Known case of right breast cancer, no available previous films for comparison.***

Findings:



- Heterogeneously dense right breast yet no definite lesions for sonomammography correlation.
- Few prominent right axillary lymph nodes are noted, the largest measuring about 2.4x1.2 cm.
- No sizable pulmonary nodules, masses or consolidative patches.
- No sizable hilar or mediastinal lymph nodes.
- Patent tracheo-bronchial tree.
- Normal CT appearance of the mediastinal vessels.
- Average size of the heart.
- No evidence of pleural or pericardial effusion.

OPINION:

- ***Heterogeneously dense right breast yet no definite lesions for sonomammography correlation.***
- ***Few prominent right axillary lymph nodes.***

Marina Essam, MSc
Dr.Noura Nabil,MD

MULTI-SLICE POST CONTRAST CT SCAN OF THE ABDOMEN AND PELVIS: -

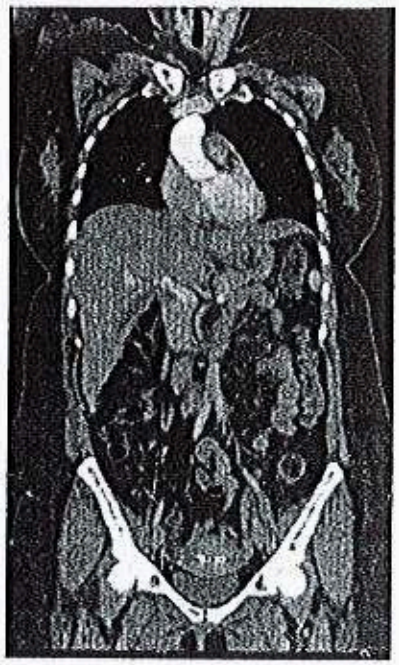
CT Scan
[[DOCTYPE:CT_SCAN]]

Patient Name: Samah Ibrahim Abd Al Tawab Sayed	Date : 09/08/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 45 Year(s)
Medical ID: 1919/2025	Pat. ID: 99892

Clinical History:

- ***Known case of right breast cancer, no available previous films for comparison.***

Findings:



- The liver is of average size (19.3 cm) with homogenous CT texture. No sizable focal lesions.
- Patent homogenously enhancing portal vein and is main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size (10.3 cm) and parenchymal texture of the spleen with no focal lesions seen.
- Normal size and parenchymal thickness of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both suprarenal glands.
- Average filling of the urinary bladder with no definite masses or stones.
- No ascites.
- Fat containing umbilical hernia is noted.
- IUCD is noted.
- Bilateral adnexal cysts are noted, the largets seen on the right side measuirng 18x20 mm.

Opinion:

- ***Hepatomegaly for clinical and lab correlation.***
- ***Fat containing umbilical hernia.***

Marina Essam, MSc
Dr.Noura Nabil,MD

Signature Noura Nabil
11/08/2025 6:14 PM

CT Scan
[[DOCTYPE:CT_SCAN]]

Name : Samah Ibrahim Abdul Tawab Said
Branch : Maadi
Ref.Physician Prof : Gostaf Roseh Hospital

Date : 28-07-2025 01:56
Medical No. : 162203
Ins.Company : التأمين الصحي

NILE DIAGNOSTIC CENTER

$Tc^{99}m$ MDP SKELETAL SCINTIGRAPHY

History:

- A 45-year-old lady, with history of right breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is scintigraphically free from metastases.

Much obliged
Dr. Mohammed AtaAllah, MSc
Nuclear Medicine
Dr. Alaa Nagi, MD
Consultant of nuclear medicine

Bone Scan
[[DOCTYPE:BONE_SCAN]]